

1. Administrative & Study Identification

Full Study Title / Official Title: A Randomised, Double-blind, Placebo-controlled, Phase III Study of Adjuvant Saruparib (AZD5305) in Patients With BRCAm Localised High-Risk Prostate Cancer Receiving Radiotherapy With Androgen Deprivation Therapy
Public Study Title: A Study of Metastases Free Survival With Saruparib vs Placebo Added to a Standard RT/ADT in Men With High-risk Prostate Cancer With a BRCA Mutation
Short Title/Acronym: EvoPAR-Prostate02
Protocol Number: D9727C00001
NCT Number: NCT06952803
Posting ID: 820584853935526398
Sponsor / Company Name: AstraZeneca
Investigational Product: Saruparib (AZD5305) or matching placebo
Phase of Development: Phase III
Trial Status: Recruiting
Medical Monitor: AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective
Primary Objective: To evaluate the efficacy of adjuvant saruparib versus placebo on metastasis-free survival (MFS) in men with early-stage, high-risk prostate cancer harboring a BRCA1/BRCA2 mutation who are receiving standard radiotherapy (RT) and androgen deprivation therapy (ADT).
Secondary Objectives: To evaluate overall survival (OS) and to assess the safety and tolerability of the saruparib combination therapy.

2.2 Background & Rationale While PARP inhibitors are currently approved for the treatment of patients with metastatic castration-resistant prostate cancer (mCRPC), this study investigates earlier intervention. Saruparib is a next-generation PARP inhibitor that selectively inhibits and traps PARP1. This trial aims to address the clinical need to delay or prevent the metastatic spread of cancer in men with high-risk, localized prostate cancer with a BRCA mutation by adding saruparib to the standard-of-care RT and ADT regimens.

3. Study Design & Methodology

3.1 Design Framework
Study Type: Interventional
Control Type: Placebo-controlled
Blinding: Double-blind
Randomization: Randomized (Stratified into Cohort A and Cohort B)

3.2 Planned Enrollment & Duration
Target Enrollment: Approximately 700 men (~400 in Cohort A; ~300 in Cohort B)
Number of Sites: ~300 centers across ~26 countries
Estimated Study Start: July 2025
Estimated Primary Completion: Event-driven (all

participants will be followed for survival until the end of the global study).

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male, Age ≥ 18 years. 2. Confirmed diagnosis of high-risk or very high-risk (localized/locally advanced) prostate adenocarcinoma, or high-risk biochemical recurrence (BCR) following radical prostatectomy. 3. Confirmed BRCA1 or BRCA2 gene mutation (BRCAm) by central tumor tissue testing, with no evidence of disease or disease detected only in the pelvis (M0) post-RT.

4.2 Exclusion Criteria 1. History of myelodysplastic syndrome (MDS) or acute myeloid leukemia (AML), or features suggestive of MDS/AML. 2. Prior use of a PARP-inhibitor. 3. Another active cancer within 3 years, recent major surgery, palliative radiation, or serious heart, liver, or kidney disease.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Saruparib 60 mg or a matching placebo tablet once daily. **Route of Administration:** Oral. **Duration of Treatment:** Core treatment spans 24 months continuous exposure alongside ADT.

5.2 Concomitant & Prohibited Therapy Permitted: Continuous ADT with a gonadotropin-releasing hormone (GnRH) analogue. Patients in Cohort B are additionally permitted abiraterone + low-dose prednisone (Prednisone ATC Codes: A07EA03, H02AB07; Trade names include: Rayos, Predone, Prednisone Intensol, Econosone, Prednicot, Liquid Pred, Meticorten, Orasone, Decortisyl, Deltasone, Prednicen-M, Sterapred DS, Sterapred). **Prohibited:** Other investigational treatments and prior use of PARP inhibitors.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, BRCA1/2 central tumor testing, Baseline Scans (CT/MRI, bone scan/PSMA-PET) confirming M0
Treatment	Every 4 Weeks	Clinic visits for exams, blood counts, electronic diary recording, and study drug dispensing
Follow-Up	Every 3 Months	PSA testing every 3 months. Imaging scans repeated every 6 months if PSA rises. Safety follow-up ~30 days after last dose

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Metastasis-Free Survival (MFS). **Measurement Method:** Confirmed by standard clinical imaging (CT/MRI and bone scan, or PSMA-PET) as assessed by a Blinded Independent Central Review (BICR).

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) * Safety and tolerability profiles (tracking tiredness, nausea, and low blood counts)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous evaluation of blood counts via clinic visits every 4 weeks and daily participant electronic diaries tracking fatigue, nausea, and other symptoms. **Serious Adverse Events (SAEs):** Standard 24-hour reporting to the Sponsor. **Data Monitoring Committee (DMC):** An Independent Data Monitoring Committee (IDMC) composed of experts will be convened to confirm the safety and efficacy of Saruparib + ADT (+ abiraterone) throughout the trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes; Safety Set for all adverse events. **Sample Size Justification:** Approximately 700 randomized patients adequately powered to conduct statistical analyses of MFS and OS within each cohort using a stratified log-rank test. **Handling of Missing Data:** Standard imputation methodology for time-to-event oncology endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard site monitoring routines coupled with centralized BRCA testing and central imaging reviews. **Audit Trail:** Secure eCRF entry logs, blinded centralized imaging systems (BICR), and data clarification queries.

11. Study Identifiers & Governance

Primary ID: D9727C00001 **Registry Number:** NCT06952803 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** EvoPAR-Prostate02 **Official Regulatory Title:** A Randomised, Double-blind, Placebo-controlled, Phase III Study of Adjuvant Saruparib (AZD5305) in Patients With

12. Clinical Framework (Prostate Cancer Specific)

Disease Area / Conditions: Prostate Cancer **Preferred UMLS Name:** BRCA-Mutated Prostate Cancer **Primary Condition:** High-Risk and Very High-Risk Prostate Cancer **Diagnosis Threshold:** BRCA1 or BRCA2 mutation with M0 disease detected **Medical Methodology:** Standard of Care Radiotherapy followed by Adjuvant Targeted Therapy + ADT **Optimization Target:** Delayed or prevented metastatic spread (MFS)

13. Study Procedures & Methodology

Management Pathway: Adjuvant targeted systemic pathway **Education Protocol (HCP):** Site initiation visits managing PARP inhibitor-associated AE management (e.g., cytopenias) **Education Protocol (Patient):** Electronic diary compliance training and instructions on routine oral medication **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all endpoint imaging verification

14. Observation & Follow-Up Schedule

Total Duration: Event-driven follow-up through to survival maturity **Onsite Visit Cadence:** Every 4 weeks during the 24-month active treatment phase **Remote Monitoring Frequency:** Continuous safety and e-diary data validation **Checklist Review Interval:** PSA testing every 3 months; imaging confirmed every 6 months pending PSA progression

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Insert Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Insert Name]	____	[DD-MMM-YYYY]				
	Lead Statistician	[Insert Name]	____	[DD-MMM-YYYY]			